



## جوستاف روسي انترناشيونال - مصر (EPE) \* طلب علاج أول مرة / متردد

إسم المريض ربيعاً : افتتان احمد السيد صالح  
المحافظة : الجيزة  
الحى / المدينة : بولاق الدكرور  
رقم البطاقة : 26807112101445 رقم التليفون : 01023686785  
العنوان : بولاق الدكرور الجيزة  
العمر (تاريخ الميلاد) : 11-07-1968 المهنة : ربه منزل  
تاريخ تسجيل الإستمارة : 26-09-2026

### الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك  
الإخصائي الاجتماعي  
التوقيع: (-----)  
المقرر بما فيه  
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)  
نتائج الأبحاث : (مرفق)  
التشخيص النهائي : سرطان بالثدي (مبادرة)  
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام  
التعليق على التشخيص : -----

### تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني  
الاسم: د/ندى حسن  
الاسم: د/نرمين رمضان  
الوظيفة: -----  
الوظيفة: -----  
الوظيفة: -----  
التوقيع: -----  
التوقيع: -----  
التوقيع: -----  
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.  
تعتمد صحة البيانات المرفقة  
رئيس اللجنة ومدير  
المستشفى.  
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

### أعضاء اللجنة

د. / ----- د. / ----- د. / -----

### قرار اللجنة

التشخيص النهائي : -----  
خطة العلاج : -----  
المستشفى : -----  
قسم : -----  
جهة العلاج : -----  
نفقات : -----  
ملاحظات : -----

ختم  
شعار الجمهورية

مدير عام  
مديرية الشؤون الصحية

رئيس  
المجلس الطبي المتخصص

|  |
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|  |
|--|

|                                                                           |                                                                                     |
|---------------------------------------------------------------------------|-------------------------------------------------------------------------------------|
| <b>Name:</b> افتتان احمد السيد صالح<br><b>National ID:</b> 26807112101445 |  |
|---------------------------------------------------------------------------|-------------------------------------------------------------------------------------|

|           |
|-----------|
| تقرير طبي |
|-----------|

|                  |
|------------------|
| السيد            |
| تحية طبية وبعد,, |

|                       |                       |
|-----------------------|-----------------------|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم:                 | رقم طبي:              |

|                                          |
|------------------------------------------|
| نتشرف بان نوضح فيما يلي تقريرنا عن حالته |
|------------------------------------------|

|                                             |                                             |
|---------------------------------------------|---------------------------------------------|
| تاريخ وساعة الدخول :<br>-- --:-- yyyy-mm-dd | تاريخ وساعة الخروج :<br>PM 01:17 05-07-2026 |
|---------------------------------------------|---------------------------------------------|

|                                        |
|----------------------------------------|
| وبعد الكشف والفحوصات والمتابعة وجد انه |
|----------------------------------------|

|                                                                                      |
|--------------------------------------------------------------------------------------|
| المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Supportive and palliative Treatment |
|--------------------------------------------------------------------------------------|

|                         |                                   |
|-------------------------|-----------------------------------|
| التاريخ :<br>05-07-2026 | الوقت :<br>-- --:--               |
|                         | الطبيب المعالج :<br>د/شيماء لاشين |


  
 جمهورية مصر العربية  
 بطاقة تحقيق الشخصية  
 افتتان  
 احمد السيد صالح  
 ١٠ ش السيد محمد علي من ش الثورة  
 بولاق النكرور - الجيزة  
 ٢٦٨٠٧١١٢١٠١٤٤٥  
 IG6238182


  
 ٢٠١٩/١٢ ٢٦٨٠٧١١٢١٠١٤٤٥  
 ربة منزل  
 انش  
 ناصر محمد احمد التميمي  
 مقروحة  
 البطاقة سارية حتى ٢٠٢٢/١٢/١١  

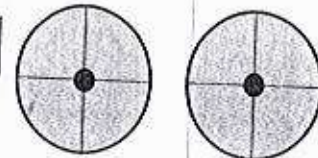
  


MDT FORM

15/1 / 2026 G

|                   |            |                |                |
|-------------------|------------|----------------|----------------|
| Name              | امينة احمد | MRN            | 1322/2026      |
| Age               | 57y        | ID             | 27A.V1151.1880 |
| Menopausal status | post       | Phone          | 01023686785    |
| NRS SCORE         |            | Marital status |                |
| BMI               |            | Family history |                |
| CO morbidity      | HTN        | REASON OF MDT  |                |

Clinical Data

|                  |        |        |       |                                                                                      |      |   |
|------------------|--------|--------|-------|--------------------------------------------------------------------------------------|------|---|
| BX:              | IDC II |        |       | R                                                                                    | ps:1 | L |
| ER:              | PR:    | HER 2: | Ki67: |                                                                                      |      |   |
| Sonomammography: |        |        |       |  |      |   |
| Mets Workup:     |        |        |       | TNM<br>Enf 3/2, pn+6/2, Her2 2+<br>3I5H: Amph 4, Ki67 25%                            |      |   |

Recommendations: Rec MAC 7phases, 6 Taxol, Carbop  
 on 24/12/2025  
 RBCS 4PTI 4PN00/5  
 for MAC  
 د. خالد عبد العزيز  
 د. منة  
 د. منة

MDT OUTCOME & PLAN

- Kadcycla (12 cycles)
- PORT
- adj AT for 5 years

SIGNATURE

|                  |         |           |           |           |             |
|------------------|---------|-----------|-----------|-----------|-------------|
| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY | Nutrition | Psychiatric |
| د. منة           | د. منة  | د. منة    |           |           |             |

Patient Name : الأستاذ / افتنان احمد السيد صالح  
Patient No. : 26807112101445  
Path No. : 3970/25  
Sex : Female  
Age : 57 Y  
Referring Doc :

Request No. : 13004148  
Request Date : 24/12/2025  
Verification Date : 10/01/2026  
Financial Category : نفقة الدولة - جوستاف روسي  
Department : Out Patient  
Registered by : Mrs.Mayada Hegazy

### Pathology Report

Clinical Data: Right breast cancer, Post-NACT.

Nature of the specimen: Right breast conservative surgery & Sentinel lymph nodes biopsy.

#### Gross Examination:

- Right breast conservative surgery specimen measured 11x8x3 cm, with no skin covering.
- Serialing showed clipped tumor bed site measured 1.8x1.5x1.5 cm, located 0.2 cm away from the least medial & inferior margins.
- Sentinel lymph nodes: Multiple fibrofatty tissue pieces collectively measured 4x4 cm, entangling 5 lymph nodes, the largest measured 2x1x0.5 cm.
- 9 Blocks:  
1:2- T.B.S., 3- Medial margin., 4- Lateral margin., 5- Superior margin., 6- Inferior margin., 7- Superficial margin., 8- Deep margin., 9- SLNs.
- Gross was done by; Dr. Mennan Hossam.

#### \*\* Frozen (Intra-operative) Results:

- Free surgical margins.
- Five free sentinel lymph nodes, SLNs (0/5).

#### Microscopic examination:

- Histological type: Residual viable invasive duct carcinoma (Actual tumour size 1.8x1.5 cm) (Cellularity 5%).
- Grade 2 (differentiation 2, nuclear pleomorphism 2, mitosis 2).
- 1% Low grade ductal carcinoma in situ (Solid pattern).
- Angioinvasion: Absent.
- Perineural invasion: Absent.
- Surgical margins: Free.
- Sentinel lymph nodes: Five free lymph nodes, (0/5).
- Histologic response to therapy: Pathologic partial response to therapy.

Verified by :

Dr.Duaa Mohamed

Printing Date : 10/01/2026 14:40:06

Page 1 of 2

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Patient Name : الأستاذ / افتنان احمد السيد صالح  
Patient No. : 26807112101445  
Path No. : 3970/25  
Sex : Female  
Age : 57 Y  
Referring Doc :

Request No. : 13004148  
Request Date : 24/12/2025  
Verification Date : 10/01/2026  
Financial Category : جوستاف روسي - نفقة الدولة  
Department : Out Patient  
Registered by : Mrs. Mayada Hegazy

### Pathology Report

#### Diagnosis:

Right breast cancer (Post-NACT), Right breast conservative surgery & Sentinel lymph nodes biopsy:

Residual viable invasive duct carcinoma, Grade 2 (Cellularity 5%) (Actual tumor size 1.8x1.5 cm).

- 1% Low grade ductal carcinoma in situ (Solid pattern).
- Surgical margins: Free.
- Sentinel lymph nodes: Five free sentinel lymph nodes, (0/5).
- Stage: ypT1c, ypN0 (sln).
- RCB: 1.352, RCB class-I.

Prof. Dr. Gamal Abd-El-Aty.  
Dr. Mennan Hossam.

Signature

Verified by :  
Dr. Duaa Mohamed

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|                 |                                     |                    |                                  |
|-----------------|-------------------------------------|--------------------|----------------------------------|
| Patient Name    | : الأستاذ / الدكتور أحمد السيد صالح | Request No.        | : 13005542                       |
| Patient No.     | : 26807112101445                    | Request Date       | : 09/04/2025                     |
| Path No.        | : 1241/25                           | Verification Date  | : 30/04/2025                     |
| Sex             | : Female                            | Financial Category | : اورام دار السلام - نفقة الدولة |
| Age             | : 56 Y                              | Department         | : Out Patient                    |
| Referring Doc : |                                     | Registered by      | : Mrs.Mayada Hegazy              |

### Pathology Report

Clinical Data: Right breast mass.  
Nature of Specimen: Tru-cut biopsy.

#### Gross Examination:

- Multiple yellowish tissue cores up to 1.2 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

#### Microscopic Examination:

- \* Histological type: Invasive duct carcinoma.
- \* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II :
- \* Ductal carcinoma in situ: Not detected.
- \* Perineural invasion: Not detected
- \* Angiolymphatic invasion: Not detected
- \* Microcalcifications: Not detected
- \* IHC (Positive control showed appropriate reactivity):
  - ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
  - PR: Strong nuclear reaction in 30% of tumor cells (score 6/8).
  - Her2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).
  - SISH: Amplified with small clusters.
- Average Her2 copy No. > 6, Her2/CEP17 ratio >4.
- Ki67: 25% nuclear reaction of tumor cells.

#### Diagnosis:

Right breast mass, Tru-cut biopsy:

Verified by :  
Dr.Toka Medhat

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CAP

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Patient Name : الأستاذة / الدكتور أحمد السيد صالح  
Patient No. : 26807112101445  
Path No. : 1241/25  
Sex : Female  
Age : 56 Y  
Referring Doc :

Request No. : 13005542  
Request Date : 09/04/2025  
Verification Date : 30/04/2025  
Financial Category : أورام دار السلام - فئة الدولة  
Department : Out Patient  
Registered by : Mrs.Mayada Hegazy

### Pathology Report

Invasive duct carcinoma, Grade II,  
IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 6/8).
- Her2: Equivocal (score 2+).
- SISH: Amplified with small clusters
- Ki67: Positive in 25% of tumor cells.

• Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

• Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, Intense < 10% of tumor cells.

Signature  
Dr.Rana Fathy.  
Dr.Duaa Mohammed.  
Dr.Samla Ali.

Verified by :  
Dr.Toka Medhat

Printing Date : 30/04/2025 15:55:28

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|                   |                                            |
|-------------------|--------------------------------------------|
| Patient ID: 80514 | Patient Name English: Aftolan Ahmed Alsaid |
| الاسم، سنة: Age:  | 2025/04/08 02:27:10 PM :Report Date        |

Name: Eftenan Ahmed Alsayed Saleh  
Date: 08-04-2024

**Women's Health Outreach Program**

56 yrs

**Bilateral Full field Digital Mammography**

**Clinical summary:** A 56 years old lady .

**Technique**

Medio-lateral oblique and CC views High resolution ultrasound

**Detailed Findings**

**Right breast**

- Heterogeneously dense breast parenchyma that may obscure small lesions (ACR density a).
- An UOQ dense spiculated lesion is noted .
- No asymmetry, distortion or suspicious clusters of microcalcifications.

**Ultrasound Right breast:**

- Preserved fibro glandular parenchyma.
- No cystic lesions.
- The right UOQ shows a hypoechoic spiculated lesion seen 10-11 o'clock zone b measures 35x30mm in dimensions with no vascularity could be seen within by color Doppler 2cm from the skin.
- Normal skin/nipple/areolar complex.
- Non-specific axillary LNs.

**Left breast**

- Heterogeneously dense breast parenchyma that may obscure small lesions (ACR density A).
- No asymmetry, distortion or suspicious clusters of microcalcifications.

**Ultrasound left breast:**

- Preserved fibro glandular parenchyma.
- No other solid or cystic lesions.
- Normal skin/nipple/areolar complex.
- Non-specific axillary LNs.

**Impression:**

Right breast (BIRADS 5): suspicious Right UOQ lesion

Left breast (BIRADS 1): normal.

Recommendation: Tru Cut core needle biopsy was done on the same setting.

Best regards,  
Dr. Omneya Ahmed.MD



|                                            |                  |
|--------------------------------------------|------------------|
| Patient Name: Eftolah Ahmed Al-Sayed Salah | Date: 08/08/2026 |
| Exam: CHEST WITH CONTRAST                  | Age: 67 Year(s)  |
| Medical ID: 99391                          | Pat ID: 99391    |

**POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:**

- Reported case of breast cancer.

**I Chest:**

- Right breast UOQ Irregular mass measuring 2.5 X 3 cm.
- No sizable pulmonary nodules, masses, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

**II Abdomen & pelvis:**

- Average sized liver showing homogenous parenchymal attenuation. No sizable focal lesions.
- No Intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, kidneys, suprarenal glands, aorta and IVC.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- IUD is seen.
- D11 lytic lesion with striations mostly hemangioma.

**Opinion:**

**Reported case of breast cancer revealed:**

- No sizable pulmonary or hepatic metastatic deposits.

*Dr. Mohamed Mofisen, M.D.*

PATIENT NAME: Allan AHMED ELSAYYED SALEH

GENDER: Female

DATE : 20-May-2025 12:15 PM

PATIENT ID: 20225019316

AGE: 56 y

ORDER: 3387180

**99MTC-MDP BONE SCAN****Desired Information: -**

- Female presenting with right breast mass: for skeletal survey.

**Technique:**

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

**Findings:**

- No hot/cold focal areas of abnormal tracer accumulation that would account for osseous deposits could be detected all over the surveyed skeleton.
- Degenerative changes are seen over dorso-lumbar vertebrae.
- Arthritic changes are seen over both knees.
- Both kidneys are normally visualized.

**Conclusion:**

- No convincing scintigraphic evidence of metastatic bone lesions all-over the surveyed skeleton.

Reported By,

Dr. ElShaymaa Hussein, Nuclear Medicine Consultant

20-May-2025 12:43 PM

22/07/2025

(10:17)

ALLAN AHMED ELSAYYED SALEH, Female, 56 y

3387180

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